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# Closing the Gap: Expanding Access to Mifepristone for Early Pregnancy Loss Through Operational Innovation

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## Abstract

Management of early pregnancy loss (EPL) with mifepristone plus misoprostol is the evidence-based standard of care endorsed by the American College of Obstetricians and Gynecologists. This 2-drug regimen demonstrates superior outcomes compared to misoprostol alone or expectant management, including fewer complications, higher success rates, and reduced need for surgical intervention. Though mifepristone use for EPL management is legal in all 50 states, it is vastly underutilized in emergency departments (EDs), which see a substantial percentage of patients with EPL. This is partly due to regulations around prescribing and dispensing it that create logistical barriers, especially in unscheduled care settings.

Expanded access to mifepristone in the ED has the potential to reduce disparities in medical care for EPL, especially for patients in rural or underserved areas. Medical therapy for EPL is often mischaracterized as elective abortion, which it is not. This concept paper defines the role of emergency clinicians in prescribing evidence-based medical treatment exclusively for patients with EPL. As restrictive state laws begin to misclassify EPL management as elective abortion care, EDs must be equipped to deliver evidence-based patient care with clarity. Protecting access to guideline-supported medical care for EPL can reduce morbidity and mortality and prevent delays in appropriate care caused by geographic, legal, or institutional barriers.

This paper proposes 5 adaptable care models for integrating mifepristone into emergency and urgent care workflows, from on-site prescribing and dispensing to hybrid pathways that incorporate telehealth and follow-up referrals. Each model addresses different levels of institutional readiness, state legal

*abstract continues*

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**Abstract (continued)**

frameworks, and clinical capacity. Implementation guidance includes protocols for eligibility screening, prescribing and dispensing options, and follow-up.

Keywords: *emergency service, hospital, pregnancy, spontaneous abortion, mifepristone, hospital administration, reproductive health services, telemedicine*

# 1 INTRODUCTION

## 1.1 Management of Early Pregnancy Loss

Early pregnancy loss (EPL), also known as spontaneous abortion or miscarriage, and defined as a nonviable intrauterine gestation of less than 13 weeks, accounts for over 900,000 emergency department (ED) visits a year.[1] There are 3 options for management of EPL—expectant, medical, and procedural. Medication management is within the scope of emergency medicine practice. Before 13 weeks of gestation, the evidence-based protocol for medication management of EPL is mifepristone 200 mg orally, followed in 24 hours by misoprostol 800 µg, which can be given vaginally, buccally, or sublingually.[2,3] Randomized trials and large cohort studies have shown that hemodynamically stable patients experiencing EPL have the best outcomes, including complete uterine evacuation and fewer return visits or surgical intervention, when treated with this regimen.[3–5] Combination therapy with mifepristone and misoprostol has been recommended by the American College of Obstetrics and Gynecology (ACOG) since 2018.[6] In a secondary analysis of the National Hospital Ambulatory Medical Care Survey (NHAMCS) from 2006 to 2016, less than 1% of EPL cases diagnosed in the ED were managed with mifepristone.[7] More recent data from 2023 suggests that patients treated in the ED are less likely to be medically managed than those seen in the outpatient setting.[4]

Mifepristone remains a guideline-recommended treatment for EPL and is legal in all 50 states. However, regulatory barriers, such as pharmacy certification, in-person dispensing mandates, and institutional hesitancy, continue to limit mifepristone dispensing and prescribing in emergency settings.[8] We present operational models that address these complexities, whereby mifepristone can be prescribed and dispensed in all 50 states for the purpose of EPL.

## 1.2 Mifepristone and the Risk Evaluation and Mitigation Strategy Program

Mifepristone, a progesterone and glucocorticoid receptor antagonist, was first developed in France in the 1980s.[9] Its initial indication was for medication abortion and was approved by the US Food and Drug Administration (FDA) in 2000 with a restricted distribution program, now known as Risk Evaluation and Mitigation Strategy (REMS).[8] In 2012, it was also approved for glycemic control in those with Cushing syndrome, the only drug of its kind on the market.[10] In 2018, it became the standard of care in combination with misoprostol for the medication management of EPL.[2,3,6]

Mifepristone must be distributed in compliance with REMS regulations regardless of the clinical indication. Under the REMS restrictions, mifepristone can only be prescribed by certified healthcare physicians who have completed a prescriber agreement form and are able to accurately date pregnancies and diagnose ectopic pregnancies. Implementation of REMS requirements varies by hospital and health system, and in some settings, certification is limited to specific specialties. The medication must be dispensed by pharmacies that are also certified under the REMS program. Patients must then sign a Patient Agreement Form confirming that they understand the treatment process and risks of the medication, as well as receive a fact sheet about mifepristone.[8]

Under current FDA regulations, mifepristone may be dispensed directly to patients or mailed from a certified hospital or retail pharmacy, without requiring in-person dispensing, without the need for an in-person visit. This federal policy supersedes the growing number of state laws that seek to mandate in-person dispensing.[11] However, recent efforts have emerged to reverse this FDA approval by requiring that mifepristone be limited to in-person clinical settings only. If successful, such federal restrictions would apply uniformly across all 50 states, further limiting access regardless of medical indication.

These layered requirements create barriers to medical care, particularly for vulnerable populations, who are more likely to face challenges related to physician availability, health literacy, and healthcare access.[12]

## 1.3 Emergency Care for Early Pregnancy Loss

EDs increasingly serve as the primary point of care for patients experiencing EPL because EPL often occurs prior to the initiation of routine prenatal care.[1] This paper presents 5 adaptable care models for integrating mifepristone access into unscheduled care settings, including EDs, urgent care, and telehealth-supported pathways. These models are based on clinical guidelines and are designed to align with legal, institutional, and resource constraints. Adherence to evidence-based guidelines for medication management of pregnancy complications can enhance the safety, quality, and equity of care.

# 2 PATIENT ELIGIBILITY AND SCREENING

Mifepristone in combination with misoprostol is a guideline-recommended treatment for EPL in patients with a confirmed nonviable intrauterine pregnancy under 13 weeks' gestation (see Figure S1 for an algorithm of management of first-trimester bleeding from the Reproductive Health Access Project). This indication is distinct from elective medication

**TABLE 1.** First-trimester pregnancy emergencies and mifepristone use.

| Indication (medical term) | Indication (colloquial term) | Clinical findings (ultrasound or laboratory findings expected) | Is mifepristone indicated? |
|---|---|---|---|
| Threatened abortion | Vaginal bleeding in early pregnancy, but pregnancy is still viable | - Vaginal bleeding<br>- Closed cervical os<br>- Ultrasound: viable intrauterine pregnancy with cardiac activity | ✕ Not indicated (pregnancy is viable; goal is expectant management) |
| Inevitable abortion | Miscarriage is definitely going to happen | - Vaginal bleeding<br>- Open cervical os<br>- Often cramping<br>- Ultrasound: intrauterine pregnancy, but cervix dilated and passage inevitable | ✅ May be indicated (medical management with mifepristone + misoprostol is effective if stable; -OB/Gyn may suggest miso only; surgical evacuation is also an option) |
| Missed abortion | Pregnancy stopped developing, but the body has not expelled tissue | - No bleeding or minimal spotting<br>- Closed cervical os<br>- Ultrasound: nonviable intrauterine pregnancy (no cardiac activity, abnormal sac, or growth arrest) | ✅ Indicated (preferred regimen: mifepristone + misoprostol) |
| Incomplete abortion | Some pregnancy tissue has passed, but not all | - Vaginal bleeding, sometimes heavy<br>- Open cervical os<br>- Ultrasound: retained products of conception in the uterus | ✅ May be indicated (medical management with mifepristone + misoprostol is effective if stable; -OB/GYN may suggest miso only; surgical evacuation is also an option) |
| Complete abortion | Miscarriage has fully occurred | - Vaginal bleeding resolves<br>- Closed cervical os<br>- Ultrasound: empty uterus, endometrial stripe only<br>- hCG declining appropriately | ✕ Not indicated (no remaining tissue; supportive care only) |
| Elective medication abortion | Ending an ongoing pregnancy | - Positive pregnancy test | ✅ Indicated (but not under EPL protocol; depends on local laws and ED policies) |

OB/Gyn, obstetrician-gynecologist.

abortion, which involves the termination of an ongoing viable pregnancy. Eligibility for mifepristone use is described in detail in Table 1. See Figure S2 for patient-facing resources comparing management strategies from the Reproductive Health Access Project. Diagnosis can be made on transvaginal ultrasound findings alone or through a combination of history, dating the last menstrual period, and physical exam demonstrating an open cervical os.[13]

Mifepristone is a safe and effective medication for most patients experiencing EPL. A comprehensive treatment algorithm is available in Figure S3 from Access Bridge. Eligible patients are hemodynamically stable, able to comprehend instructions, and capable of accessing follow-up care. Absolute contraindications include suspected or confirmed ectopic pregnancy, chronic adrenal insufficiency, long-term corticosteroid use, coagulopathy or therapeutic anticoagulation, active pelvic infection or sepsis, and known hypersensitivity to mifepristone or misoprostol.[13,14] Intrauterine devices should be removed before initiating treatment.[14]

Patients should be counseled on the expected course of medical management, potential complications, and clear follow-up plans. A posttreatment assessment may include outpatient clinic visits or telehealth services. Standard discharge instructions should be provided, including specific guidance on when to return to the ED for symptoms such as fever, severe pain, or clinically significant bleeding. Ongoing follow-up can occur through local obstetric clinics or telehealth to ensure safe and effective wrap-around care.[15]

The following care models represent operational protocols that can be adapted based on local policies, clinician training, and available resources (summarized in Table 2).

## 3 MODELS FOR PRESCRIBING AND DISPENSING MIFEPRISTONE IN UNSCHEDULED CARE

### 3.1 Prescribing and Dispensing of Mifepristone During a Single ED Visit

Upon diagnosis of EPL, mifepristone can be administered in the ED, along with a prescription for misoprostol to be taken at home 24 to 48 hours later. This model ensures timely initiation of treatment and may be particularly valuable in

**TABLE 2.** Five care models: key features and challenges.

| Model | Description | Key features | Challenges/considerations |
|---|---|---|---|
| 1. Prescribing and dispensing during a single ED visit | Mifepristone given in the ED + prescription for misoprostol to take at home 24–48 h later. | - Ensures timely initiation of treatment.<br>- Especially useful in rural or abortion-restricted regions.- Requires ED formulary inclusion + REMS-certified prescriber.<br>- Implementation support available via programs like Access Bridge. | - Requires hospital policy approval and staff training.<br>- Limited by REMS certification access.<br>- Institutional/legal review may be necessary. |
| 2. Prescribing in ED, dispensing at an outpatient pharmacy | Mifepristone is prescribed in ED, filled at a certified outpatient or retail pharmacy. | - REMS-certified physician (ED or OB-GYN) confirms eligibility.<br>- Prescription sent to REMS-certified retail pharmacy.<br>- Patient controls timing of initiation, supporting flexibility (work, childcare, transport). | - Pharmacy REMS certification is a bottleneck.<br>- Limited number of certified physicianss.<br>- May delay initiation if pharmacy access is constrained. |
| 3. Facilitated referral to in-person outpatient care | If mifepristone is unavailable in ED, arrange urgent outpatient referral for combined therapy; if not feasible, offer misoprostol-only regimen or expectant management. | - Goal referral window: 24–48 h.<br>- Misoprostol-only acceptable if mifepristone not available (ACOG-supported).<br>- Better outcomes than expectant management. | - Misoprostol-only regimen is less effective than combined therapy.<br>- More complications and prolonged symptoms compared to the standard regimen.<br>- Requires coordinated referral pathways. |
| 4. Facilitated referral to telehealth care | ED refers patient to REMS-certified telehealth physician who prescribes mifepristone/ misoprostol and arranges pharmacy or mail-order dispensing. | - Leverages telehealth infrastructure.<br>- Improves access when local physicians are unavailable.<br>- Evidence supports telehealth EPL follow-up as safe/effective.<br>- Maintains continuity across settings. | - Requires reliable internet/tech access for patient.<br>- Not always standard in ED workflows.<br>- Dependent on telehealth pharmacy network availability. |
| 5. Outpatient/ telehealth referral to ED | Outpatient or telehealth visit identifies eligible patient, then refers to ED for mifepristone initiation (model 1 or 2). | - Expands ED's role as backup access point.<br>- ED can serve patients who cannot get outpatient/telehealth follow-up quickly.<br>- Promotes awareness that ED can provide EPL care. | - Requires ED outreach and education to outpatient networks.<br>- May be underutilized if outpatient physicians do not know ED offers this care.<br>- Relies on existing ED workflows. |

rural or abortion-restricted regions where outpatient pharmacy access to mifepristone is limited. In this model, a REMS-certified prescriber within the ED is identified, and mifepristone is included on the ED formulary, which may involve institutional review, administrative approval, and staff training. Initiatives such as Access Bridge provide materials for ED-based implementation strategies and clinician training.[14]

The authors have participated in the implementation of this model of care at a large tertiary care hospital in a state where elective abortion is prohibited by law. In this case, a single emergency physician served as the REMS-certified physician on record, and all orders for mifepristone were routed to him for cosignature. Department policy did not require real-time prescribing approval as mifepristone was used exclusively for EPL according to criteria established jointly by the Departments of Emergency Medicine, Obstetrics and Gynecology, and Pharmacy, along with hospital legal counsel.

### 3.2 Prescribing Mifepristone During an ED Visit With Subsequent Outpatient Pharmacy Dispensing

Prescribing mifepristone with outpatient pharmacy dispensing following an ED visit offers an alternative to on-site medication administration. During the ED encounter, a REMS-certified clinician, either the emergency physician or a consulting obstetrician-gynecologist, confirms the diagnosis and assesses patient eligibility. Prescriptions for mifepristone and misoprostol are then either written or electronically transmitted to a retail pharmacy that is certified to dispense mifepristone.

Barriers to implementation remain constrained by the REMS requirements; in many institutions, certification is limited to a small number of physicians, typically within obstetrics and gynecology, thereby creating logistical challenges for ED-based prescribing. Alternatively, this model allows patients to determine the timing of treatment initiation, which may be particularly valuable for individuals who must coordinate work obligations, childcare, or transportation. This flexibility supports patient-centered care while maintaining alignment with clinical guidelines and safety protocols.

### 3.3 Facilitated Referral to In-Person Outpatient Care Model

If mifepristone is not available on the ED formulary and a REMS-certified prescriber is not available during the visit, clinicians have 3 options: (1) expedited referral for mifepristone-misoprostol combined therapy, (2) initiating a misoprostol-only regimen from the ED, or (3) expectation management. Referral should be arranged on an urgent basis with a target access window of approximately 24 to 48 hours. Although the combined regimen is the preferred standard, the ACOG also recognizes misoprostol-only management as a safe and acceptable alternative when mifepristone cannot be accessed. A misoprostol-only protocol has lower efficacy,[16] increases the risk of complications, prolongs symptom burden compared to the combined regimen, and is no longer considered standard of care if mifepristone is available.[13] However, it is preferable to expectant management, and misoprostol alone remains evidence-based and may be appropriate when referral is not feasible or would cause undue delays in care.

We have an opportunity to improve outcomes by coordinating urgent outpatient referral pathways for mifepristone initiation with REMS-certified outpatient physician. This staged care model offers a pragmatic solution in resource-constrained or hospital policy-restricted settings, where direct access to mifepristone in the ED is unavailable.

### 3.4 Facilitated Referral to Telehealth Care Model

The expansion of telehealth services has significantly improved access for a variety of patient groups in diverse clinical settings and serves as a new, viable option for referral from the ED for EPL management. Although telehealth follow-up is not a traditionally established referral pathway from the ED, it may offer a feasible and effective alternative in cases where timely in-person follow-up is not possible. The REMS-certified telehealth clinician reviews documentation from the ED visit, confirms eligibility, and prescribes mifepristone and misoprostol. Under current FDA guidance, the medication may be dispensed via a REMS-certified mail-order pharmacy or sent to a local pharmacy for in-person pick-up.

This model leverages existing telehealth infrastructure to extend the reach of evidence-based reproductive care and promote continuity between emergency and outpatient services. Telehealth follow-up after EPL management has been demonstrated to be a feasible and effective alternative to in-person care, with similar rates of follow-up completion and no increase in complications.[15]

### 3.5 Outpatient or Telehealth Care Referral to ED

In this model, an outpatient or telehealth visit serves as the initial point of contact to evaluate patient history and whether the patient is eligible for treatment with mifepristone and misoprostol. If timely outpatient follow-up is unavailable and the telehealth physician is not REMS-certified, the patient may be referred to the ED for further evaluation and initiation of medication management. Depending on the workflow of the ED, eligible patients could receive mifepristone in the ED as outlined in model 1 or receive a prescription as outlined in model 2. The viability of this model for improving EPL management requires that EDs actively outreach to promote that this care can be provided from the ED, as this may not have been previously provided and therefore would not be known in the outpatient setting.

## 4 IMPLEMENTATION CONSIDERATIONS

Implementing protocols to administer mifepristone in unscheduled care settings requires institution-specific planning and staff education. EDs should develop standardized protocols that include eligibility checklists, counseling scripts, documentation templates, and referral pathways. Training must encompass not only the clinical use of mifepristone and misoprostol but also legal frameworks and strategies for risk mitigation in restrictive policy environments.[17]

Billing and reimbursement pathways should also be clearly defined to ensure that services are financially sustainable.

## 5 ETHICAL AND LEGAL IMPLICATIONS

Providing timely management of early pregnancy loss in emergency settings has important clinical benefits, including reduced pain, shorter duration of bleeding, decreased psychological distress, and lower risk of maternal morbidity and mortality. However, clinicians practicing in legally restrictive states may face institutional barriers to care or legal risks that require additional safeguards.[18]

Risk management strategies include partnering with legal experts, implementing clear documentation practices, and establishing telehealth linkages to external physicans where appropriate.

## CONCLUSION

A significant gap remains in ED utilization of mifepristone for EPL, representing a missed opportunity to prevent avoidable patient harm. The Dobbs decision, subsequent state-level legislation, and the associated public discourse have created a false impression that providing appropriate care for EPL is equivalent to providing elective terminations of pregnancy. This has unnecessarily led to the withholding of appropriate medical care for women experiencing a common medical condition, which in turn leads to preventable morbidity and mortality. This paper presents 5 practical, proven care models that EDs can adopt to expand timely access to mifepristone. Implementing these approaches is critical to ensuring equitable, guideline-based reproductive care across diverse legal and resource settings.

## DECLARATION OF GENERATIVE AI AND AI-ASSISTED TECHNOLOGIES IN THE MANUSCRIPT PREPARATION PROCESS

During the preparation of this work, the authors used ChatGPT-40 in order to improve clarity of writing and reduce repetitive wording. After using this tool/service, the authors reviewed and edited the content as needed and take full responsibility for the content of the publication.

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By *JACEP Open* policy, all authors are required to disclose any and all commercial, financial, and other relationships in any way related to the subject of this article as per ICMJE conflict of interest guidelines (see www.icmje.org). The authors have stated that no such relationships exist.

## CONFLICT OF INTEREST

All authors have affirmed they have no conflicts of interest to declare.

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## SUPPLEMENTARY MATERIALS

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